



Results

BIOPSY OR SURGICAL SPECIMEN (Order [REDACTED])

Patient Information

Patient Name [REDACTED]

MRN [REDACTED]

Sex
Male

DOB [REDACTED]

Result Information

Status

Final result [REDACTED]

Provider Status
Ordered

Entry Date

Component Results

Component

Surgical Pathology

[REDACTED]
[REDACTED]

Final Diagnosis

A. Right distal ureter, margin segment, cystoprostatectomy:

- Benign ureter segment
- Frozen section diagnosis confirmed

B. Left distal ureter, margin segment, cystoprostatectomy:

- Benign ureteral segment
- Frozen section diagnosis confirmed

C. Bladder and prostate, cystoprostatectomy:

- Invasive, high grade urothelial carcinoma with prominent, extensive squamous differentiation (see note)
- Carcinoma is ulcerated, involves about 80 percent of the bladder wall including the intravesical right ureter
- Largest tumor nodules is 7.5 x 4.3 x 0.7 cm in the right lateral wall; multiple other tumor nodules identified in fundus, left lateral wall, anterior wall and in the prostate (2.5 cm)
- Tumor infiltrates through the muscularis propria, extensively into the perivesical adipose tissue, into the prostatic stroma (right greater than left) and right seminal vesicle (blocks C5 and C6)
- Extensive perineural invasion present
- No lymphovascular invasion identified
- Carcinoma is present at the posterior serosal surface (block C26) and at the right anterior prostatic capsular resection margin (block C31)
- Bilateral ureteral and urethral resection margins are negative for carcinoma
- Benign bladder mucosa with extensive squamous metaplasia, chronic cystitis with reactive lymphoid follicles; diverticulum in the right fundus with squamous metaplasia

D. Left pelvic lymph nodes, dissection:

- Three lymph nodes with metastatic carcinoma
- Extranodal extension present (3/3)

E. Right pelvic lymph nodes, dissection:

- One of seven lymph nodes with metastatic carcinoma (1/7)

ICD-O-3

Carcinoma, urothelial NOS
8120/3

Site Bladder NOS
067.9
pJ 8/13/13

Per TSS diagnostic discrepancy form,
no squamous differentiation seen on
TCGA specimen.
BCE

CAP CANCER CHECKLIST

[REDACTED]

Specimen:
Bladder and prostate

Procedure:
Radical cystoprostatectomy

Tumor Site:
Right and left lateral walls, anterior wall, posterior wall, dome of bladder

Tumor Size:
Approximate dimension: 9.0 x 5.0 x 5.0 cm

Histologic Type:
Urothelial carcinoma with extensive squamous differentiation

Associated Epithelial Lesions:
None identified

Histologic Grade:
Urothelial Carcinoma (WHO/2004 ISUP): High grade

Tumor Configuration:
Solid, interconnected nests with keratinization and acantholytic foci;
ulcerated/necrotic at the mucosal surface

Microscopic Tumor Extension:
Perivesical fat, prostatic stroma, and into the intravesical portions of the
right and left ureters

Margins:
Tumor present at the posterior vesical serosal surface (block C26), right
anterior prostatic capsular resection margin (block C31)
Ureteral and urethral resection margins, uninvolved by carcinoma

Lymphovascular Invasion:
Not identified

Perineural Invasion:
Present, extensive

Pathologic Staging (pTNM)
Primary Tumor (pT):
pT4a: Tumor invades prostatic stroma

Regional Lymph Nodes (pN):
pN2: Multiple regional lymph node metastases in the true pelvis
Number examined: 10
Number involved: 4

Distant Metastasis (pM): pMX: Cannot be assessed

Additional Pathologic Findings:
Tumor associated with prominent inflammatory cell infiltrate including
lymphocytes, plasma cells and eosinophils
Non-neoplastic urothelium including right fundic diverticulum with extensive
squamous metaplasia and chronic cystitis with reactive lymphoid follicles

Ancillary Studies:
None performed at this time; blocks C7 or C33 may be used if required

The above pathology diagnosis incorporates data elements as outlined in the CAP Cancer Protocols and Checklists which are based on the AJCC/UICC TNM, 7th edition.

Note

The tumor is ulcerated and necrotic at the bladder mucosa and is comprised of solid, interconnected nests with keratinization and acantholytic foci. There is extensive squamous differentiation. Tumor in the prostate and in the lymph nodes is a syncytium of malignant cells associated with prominent mixed chronic inflammation comprised of lymphocytes, plasma cells and eosinophils, features of lymphoepithelioma-like carcinoma.

Immunohistochemical stains performed with appropriate controls show the following results within the tumor cells:

CK7 Positive, extensive

CK20 Negative

CK5/6 Positive, extensive

AE1/AE3 Positive, extensive

LCA highlights leucocytes at the edges of the tumor cell nests/syncytium

EBV in-situ hybridization negative

but no differentiation was seen in piece taken for TCGA

This is a high-grade carcinoma with extensive squamous differentiation. While a high-grade urothelial carcinoma with squamous differentiation is favored, squamous tumor histomorphology, negative CK20, extensive CK5/6 expression, and extensive benign squamous metaplasia raise the possibility of a de-novo squamous cell carcinoma.

Selected slides are reviewed at the [REDACTED]

The findings are communicated via e-mail to Dr. [REDACTED]

Clinical History

Bladder cancer

Cystectomy with ureteroileal conduit

Source:

A: Right distal ureter

B: Left distal

C: Bladder and prostate

D: Left pelvic lymph nodes

E: Right pelvic lymph node

Gross Description

Specimen A, received fresh labeled with the patient's name and designated "right distal ureter" is a tubular segment of tissue, 0.3 cm in length and 0.5 cm in diameter. The specimen is submitted entirely for frozen section diagnosis as A1.

Specimen B, received fresh labeled with the patient's name and designated "left distal ureter" is a tubular segment of tissue, 0.1 cm in length and 0.4 cm in diameter. The specimen is submitted entirely for frozen section diagnosis as B1.

Specimen C

Type of specimen: Urinary bladder, ureter stumps, prostate, seminal vesicles

The patient's name and case number on the specimen container match the accompanying paperwork and cassettes.

[REDACTED]

Procedure: Cystectomy

Fixative: Fresh

Dimensions:

Bladder - 9.5 cm SI x 5.3 cm LR x 5.5 cm AP

Right ureter - 8.2 cm in length and 0.4 cm in diameter

Left ureter - 2.7 cm in length and 0.4 cm in diameter

Prostate - 3.5 cm apex to base x 4.3 cm left to right x 4.0 cm anterior to posterior

Overall dimensions - 22.0 cm SI x 12.0 cm RL x 5.5 cm AP

Description of findings:

Tumor: 80% of the bladder mucosa is covered with ulcerated tumor. In the right lateral wall, the tumor is 7.5 x 4.3 x 0.7 cm, invading through the wall and to within 0.6 cm of the inked soft tissue margin. In the left fundus, there is a 3.5 x 2.5 cm area of tumor which extends through the wall and to within 1.2 cm of the soft tissue margin. In the left lateral wall, there is a 2.5 x 2.2 cm area of tumor which extends through the wall to the inked soft tissue margin. In the anterior wall, there is a 4.5 x 4.0 cm area of tumor which extends through the wall to the inked soft tissue margin. The tumor is ulcerated and friable, most friable and ulcerated in the right lateral wall. The right ureter is adhered to the soft tissue, extending inferiorly and curving upward to the ureteral orifice.

Involvement of important adjacencies: The tumor abuts and surrounds the left and right ureteral orifice, extends into the prostate, most prominent in the right anterior base, extends into the seminal vesicles and focally extends through the wall into the soft tissue.

Uninvolved mucosa: The mucosa not involved by tumor is somewhat edematous. There is a pouch-like diverticulum in the right fundus, 2.5 cm in length with a diameter of 2.2 cm. The pouch communicates with the bladder cavity. Ulcerated tumor does not extend into the diverticulum.

Prostate: The prostate gland is serially sectioned apex to base into five slabs. In the right anterior aspect of slab 2-5, there is tan firm tumor with ulceration. The tumor is within 0.6 cm of the inked capsule. The remainder of the gland is tan and granular, with focal striations and pale foci.

Ink code:

Anterior bladder - green

Posterior bladder - black

Right prostate - orange

Left prostate blue

Gross photographs are taken

Section key:

C1 - urethral margin

C2 - left ureter and left ureteral orifice, blue ink on ureter

C3 - right ureter and right ureteral orifice, blue ink on ureter

C4-C6 - central bladder neck to seminal vesicle with tumor

C7-C8 - right bladder neck to seminal vesicle with tumor

C9-C10 - left bladder neck to seminal vesicle with tumor

C11-C13 - right lateral wall tumor to inked soft tissue margin

C14-C15 - left lateral bladder mucosa with tumor in soft tissue

C16 - left lateral wall

C17 - left fundus, tumor to inked soft tissue margin

C18-C20 - anterior bladder tumor to inked soft tissue margin

C21-C25 - diverticulum right fundus, C23-C25 junction with bladder

C26-C27 - tumor at posterior soft tissue margin

C28-C29 - left lateral wall tumor through wall to inked soft tissue margin

C30 - slab 1 prostate right apex serially sectioned

C31 - slab 2 prostate right anterior

C32 - slab 2 prostate right posterior

C33 - slab 4 prostate right anterior

C34 - slab 5 prostate right anterior

C35- slab 5 prostate right posterior
 C36 - slab 3 prostate right anterior
 C37 - slab 1 prostate left apex serially sectioned
 C38 - slab 2 prostate left anterior
 C39 - slab 2 prostate left posterior
 C40 - slab 4 prostate left anterior
 C41 - slab 4 prostate left posterior
 C42 - slab 5 prostate left posterior

Specimen D, received in formalin labeled with the patient's name and designated "left pelvic lymph node" is adipose tissue, 5.5 x 2.0 x 1.5 cm. There are three nodes, 1.2 cm, 1.5 cm, and 2.0 cm. The cut surface of each node shows a diffuse white infiltrating parenchyma. The nodes are submitted entirely as follows:

D1 - one node trisected
 D2 - one node quadrisectioned
 D3 - one node serially sectioned into six pieces

Specimen E, received in formalin labeled with the patient's name and designated "right pelvic lymph node" is adipose tissue, 5.5 x 3.5 x 1.6 cm. There are seven possible nodes, 0.5 to 2.0 cm. One of the larger nodes shows infiltrating white parenchyma (E2). The nodes are submitted entirely as follows:

E1 - four whole nodes
 E2 - one node serially sectioned into five pieces
 E3 - one node serially sectioned into five pieces
 E4 - one node serially sectioned into five pieces

Microscopic Description

The stain quality is acceptable.

A-E. The microscopic findings are reflected in the diagnoses rendered.

Intraoperative Diagnosis

Time in: Time out:
 A1-FS. Right distal ureter: "Negative for tumor."

Time in: Time out:
 B1-FS. Left distal ureter: "Negative for tumor; incomplete transection."

Reported to Dr. by Dr.

***Electronically Signed Out By

Results

Pathology Report

Intraoperative Consultation Report

Result History

BIOPSY OR SURGICAL SPECIMEN

Order Result History Report.

Collection Information

Specimen Source Date and Time
 Other

Accession #

Lab Information

Criteria	W 8/8/13	Yes	No
Diagnosis Discrepancy	Prostate - 10		✓
Primary Tumor Site Discrepancy			✓
HIPAA Discrepancy			✓
Prior Malignancy History			✓
Dual/Synchronous Primary Noted	10/28/13		✓
Case is (circle):	QUALIFIED / DISQUALIFIED		
Reviewer: Initials	MB	Date Reviewed:	8/5/13